

|              |                    |                  |                       |
|--------------|--------------------|------------------|-----------------------|
| Patient Name | : Mr.ARUN SHETTY   | Collected        | : 11/Dec/2023 07:51AM |
| Age/Gender   | : 36 Y 8 M 18 D /M | Received         | : 11/Dec/2023 10:26AM |
| UHID/MR No   | : APJ1.0017786979  | Reported         | : 11/Dec/2023 10:34AM |
| Visit ID     | : DBLBOPV6800      | Status           | : Final Report        |
| Ref Doctor   | : Dr.SELF          | Client Name      | : PUP 24X7_CREDIT     |
| IP/OP NO     | :                  | Patient location | : HINKAL,Mysore       |

**DEPARTMENT OF HAEMATOLOGY**

**APOLLO BASIC FULL BODY CHECK**

| Test Name                             | Result | Unit | Bio. Ref. Range | Method            |
|---------------------------------------|--------|------|-----------------|-------------------|
| HAEMOGLOBIN , <i>WHOLE BLOOD EDTA</i> | 13.1   | g/dL | 13-17           | Spectrophotometer |



|              |                    |                  |                       |
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| UHID/MR No   | : APJ1.0017786979  | Reported         | : 11/Dec/2023 11:01AM |
| Visit ID     | : DBLBOPV6800      | Status           | : Final Report        |
| Ref Doctor   | : Dr.SELF          | Client Name      | : PUP 24X7_CREDIT     |
| IP/OP NO     | :                  | Patient location | : HINKAL,Mysore       |

**DEPARTMENT OF BIOCHEMISTRY**

**APOLLO BASIC FULL BODY CHECK**

| Test Name                                    | Result | Unit  | Bio. Ref. Range | Method   |
|----------------------------------------------|--------|-------|-----------------|----------|
| GLUCOSE, RANDOM , SODIUM FLUORIDE PLASMA (R) | 101    | mg/dl | 70-140          | GOD, POD |



|              |                    |                  |                       |
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| Patient Name | : Mr.ARUN SHETTY   | Collected        | : 11/Dec/2023 07:51AM |
| Age/Gender   | : 36 Y 8 M 18 D /M | Received         | : 11/Dec/2023 01:27PM |
| UHID/MR No   | : APJ1.0017786979  | Reported         | : 11/Dec/2023 01:52PM |
| Visit ID     | : DBLBOPV6800      | Status           | : Final Report        |
| Ref Doctor   | : Dr.SELF          | Client Name      | : PUP 24X7_CREDIT     |
| IP/OP NO     | :                  | Patient location | : HINKAL,Mysore       |

DEPARTMENT OF BIOCHEMISTRY

APOLLO BASIC FULL BODY CHECK

| Test Name                                             | Result | Unit  | Bio. Ref. Range | Method     |
|-------------------------------------------------------|--------|-------|-----------------|------------|
| HBA1C, GLYCATED HEMOGLOBIN ,<br>WHOLE BLOOD EDTA      | 5.1    | %     |                 | HPLC       |
| ESTIMATED AVERAGE GLUCOSE (eAG) ,<br>WHOLE BLOOD EDTA | 100    | mg/dL |                 | Calculated |

**Comment:**

Reference Range as per American Diabetes Association (ADA) 2023 Guidelines:

| REFERENCE GROUP        | HBA1C %   |
|------------------------|-----------|
| NON DIABETIC           | <5.7      |
| PREDIABETES            | 5.7 – 6.4 |
| DIABETES               | ≥ 6.5     |
| DIABETICS              |           |
| EXCELLENT CONTROL      | 6 – 7     |
| FAIR TO GOOD CONTROL   | 7 – 8     |
| UNSATISFACTORY CONTROL | 8 – 10    |
| POOR CONTROL           | >10       |

**Note:** Dietary preparation or fasting is not required.

- HbA1C is recommended by American Diabetes Association for Diagnosing Diabetes and monitoring Glycemic Control by American Diabetes Association guidelines 2023.
- Trends in HbA1C values is a better indicator of Glycemic control than a single test.
- Low HbA1C in Non-Diabetic patients are associated with Anemia (Iron Deficiency/Hemolytic), Liver Disorders, Chronic Kidney Disease. Clinical Correlation is advised in interpretation of low Values.
- Falsely low HbA1c (below 4%) may be observed in patients with clinical conditions that shorten erythrocyte life span or decrease mean erythrocyte age. HbA1c may not accurately reflect glycemic control when clinical conditions that affect erythrocyte survival are present.
- In cases of Interference of Hemoglobin variants in HbA1C, alternative methods (Fructosamine) estimation is recommended for Glycemic Control
  - HbF >25%
  - Homozygous Hemoglobinopathy.
 (Hb Electrophoresis is recommended method for detection of Hemoglobinopathy)



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| UHID/MR No   | : APJ1.0017786979  | Reported         | : 11/Dec/2023 11:31AM |
| Visit ID     | : DBLBOPV6800      | Status           | : Final Report        |
| Ref Doctor   | : Dr.SELF          | Client Name      | : PUP 24X7_CREDIT     |
| IP/OP NO     | :                  | Patient location | : HINKAL,Mysore       |

**DEPARTMENT OF BIOCHEMISTRY**

**APOLLO BASIC FULL BODY CHECK**

| Test Name                       | Result | Unit  | Bio. Ref. Range | Method |
|---------------------------------|--------|-------|-----------------|--------|
| <b>BILIRUBIN, TOTAL , SERUM</b> | 0.71   | mg/dl | 0-1.2           | NBD    |



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| Ref Doctor   | : Dr.SELF          | Client Name      | : PUP 24X7_CREDIT     |
| IP/OP NO     | :                  | Patient location | : HINKAL,Mysore       |

**DEPARTMENT OF BIOCHEMISTRY**

**APOLLO BASIC FULL BODY CHECK**

| Test Name          | Result | Unit  | Bio. Ref. Range | Method                       |
|--------------------|--------|-------|-----------------|------------------------------|
| CREATININE , SERUM | 1.12   | mg/dL | 0.66-1.25       | Creatinine<br>amidohydrolase |

**\*\*\* End Of Report \*\*\***

  
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